



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°123/MVEBDB/09/14/2026

Reporting date: September 14, 2026

Publication date: September 15, 2026

HIGHLIGHTS (last 24 hours)



In the past 24 hours, 87 new confirmed cases of Ebola Virus Disease (EVD) have been reported, including 26 community deaths were recorded. These cases originated from the provinces of Ituri (57 cases), North Kivu (21 cases), and Haut-Uélé (9 cases). In addition, 9 deaths within the Ebola Treatment Center (ETC) were also recorded.

✓ No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,345	ACCUMULATIVE DEATH CONFIRMED 3,545	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 938	ACCUMULATIVE HEALED 1,753	TRACKING RATES CONTACTS (Of the Day) 89.3%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



62 out of 167
health zones affected
(37.1%)

Summary of key points from the last 24 hours

Transmission remains active in three provinces, namely **Ituri (57 cases), of North Kivu (21 cases) and Haut-Uélé (9 cases)**, for a total of **87 new confirmed cases** of Ebola Virus Disease (EVD) including **26 community deaths reported** for the day of September 14, 2026. In addition, **9 intra-ETC deaths** were reported, including 4 in Ituri and 3 in North Kivu and 2 in Haut-Uélé, bringing the total number of deaths for the day to **35**.

In total, 27 patients were declared cured after obtaining two negative results for Ebola virus disease, respectively **17 in Ituri, 6 in North Kivu and 4 in Haut-Uélé**.

From the start of the epidemic until September 14, 2026, **there were 7,345 confirmed cases, including 3,545 deaths**, representing a case fatality rate of **48.3 %**. The main hotspot remains the Ituri province, which accounts for **77.8%** of the confirmed cases reported in the seven affected provinces and also records **65.5%** of new confirmed cases reported in 3 provinces.

No new health zones affected in the last 24 hours. DRC

continues to have **62 health zones out of 167 (37.1%) affected** and **7 provinces affected**. Ituri has 28 health zones affected out of 36, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 14, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	57	5716	2,604	45.6%	28/36 (77.8%)
North Kivu	21	1302	808	62.1%	16/34 (47.0%)
Haut-Uélé	9	290	118	40.7%	6/13 (46.1%)
Tshopo	0	28	10	35.7%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	5	3	60.0%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	87	7,345	3,545	48.3%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 14, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5716	2604	45.6%	57	17	4	21
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	179	39	21.8%				0
Boga	2	2	100.0%				0
Bunia	1603	508	31.7%	20	3		3
Damascus	39	9	23.1%				0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	136	88	64.7%	1	0		0
Lita	264	130	49.2%	2	0		0
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	56	31	55.4%	9	6		6
Mangala	315	146	46.3%	2	0		0
Mongbwalu	666	305	45.8%	3	2		2
Nia-Nia	245	126	51.4%	6	6		6
Nizi	737	273	37.0%	5	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

Rwampara	1026	349	34.0%	7	0		0
Tchomia	73	25	34.2%	2	0		0
To ventilate *	N / A	446	N / A			4	4
North Kivu	1302	808	62.1%	21	8	3	11
Blessed	231	170	73.6%	3	2	2	4
Good	18	7	38.9%	1	0		0
Butembo	240	157	65.4%	1	0		0
Goma	1	0	0.0%				0
Kalunguta	32	12	37.5%	2	0		0
Katwa	533	337	63.2%	10	4	1	5
Kayna	1	0	0.0%				
Kyondo	30	19	63.3%				0
Lubero	15	7	46.7%				0
Mabalako	19	9	47.4%				0
Manguredjipa	9	3	33.3%	1	0		0
Masereka	21	9	42.9%	1	1		1
Musienene	117	64	54.7%	1	1		1
Mutwanga	6	2	33.3%				0
Oicha	11	7	63.6%				0
Vuhovi	18	5	27.8%	1	0		0
Haut-Uélé	290	118	40.7%	9	1	2	3
Boma Mangbetu	34	15	44.1%	2	1		1
Gombari	3	1	33.3%				0
Isiro	98	42	42.9%	1	0	1	1
Pawa	61	26	42.6%	3	0	1	1
Rungu	2	1	50.0%				0
Wamba	92	33	35.9%	3	0		0
Tshopo	28	10	35.7%	0	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	4	2	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	5	3	60.0%	0	0	0	0
Buta	1	1	100.00%				0
Viadana	1	0	0.00%				0
Ganga	3	2	66.67%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7345	3545	48.3%	87	26	9	35

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province, as of September 14, 2026

Table 1: Details of alerts by category, province, and type of perpetrator, 2019									
DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	18	1	19	11	1	7	0	12	11
Bas-Uélé	35	0	35	2	0	33	0	2	0
Ituri	730	69	799	135	61	361	4	191	112
North Kivu	1024	57	1081	137	57	672	0	194	83
South Kivu	6	0	6	2	0	3	0	2	1
South Ubangi	ND	ND	N / A	ND	ND	ND	ND	ND	N / A
Tshopo	82	0	82	7	0	75	0	7	0
Total General	1895	127	2022	294	119	1,151	4	408	207

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

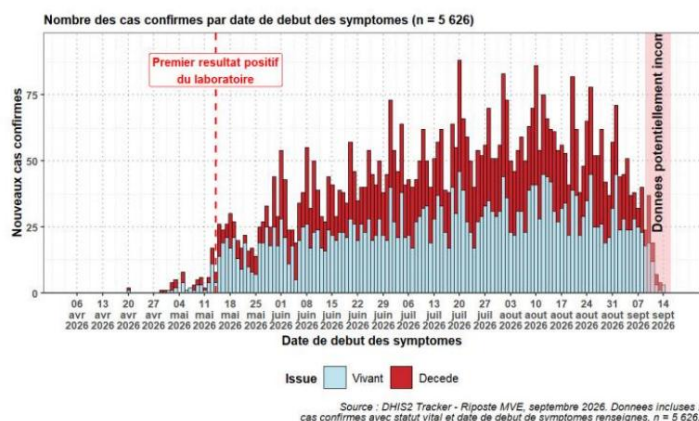
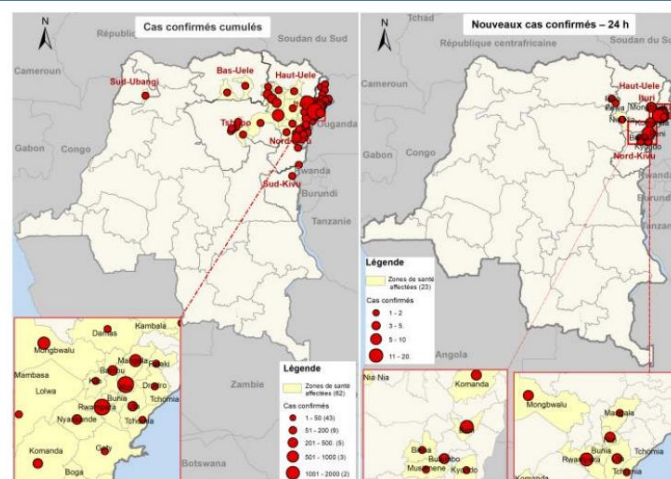


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Of the seven affected provinces, six (Sud Ubangi did not report) submitted alert management data. A total of **2,022 alerts** were recorded on September 14, 2026, of which 1,568 (77.5%) were verified. Following this verification process, **413 (26.3%) alerts were validated as suspected cases**, of which 408 were investigated, and **207 of these were referred to the CT/CTE (Treatment Centers/Emergency Treatment Centers)**. (Table 3).

The proportion of contacts followed up is 89.3% (31,389/35,158), reflecting an improvement compared to the previous day (78.6%) and above the 85% threshold; 91.4% in Ituri (19,126/20 930), 86.9% in North Kivu (10,854/12,490), 83.1% in Haut-Uélé (1,025/1,233); 79.3% in Tshopo (222/280); 78.5% in Bas-Uélé (146/186) and 41.0% in Sud Ubangi (16/39).

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Four (4) alerts were notified to the PoE/PoC in the last 24 hours, three (3) in Ituri and one (1) in North Kivu. In Ituri, an alert at the Dele PoC (Rwampara Health Zone) was validated, but the patient refused transfer; a body was intercepted at the Uguro PoC (Mahagi Health Zone), the alert was validated, but the body was not swabbed; another body traveling from Bunia to Kasenyi was intercepted.

Also at the Mandje PoC (Tchomia Health Zone), a swab test was conducted, and the result was negative. In North Kivu, a lifeless body traveling from Butembo to Mangina was intercepted at the Mukulia PoC (Beni Health Zone), notified, investigated, and referred for swab testing.

Table 4. Status of key PoE/PoC indicators by province, as of September 14, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.7% (58/60)	94.7% (18/19)	55.6% (5/9)	44.4% (8/18)	63.6% (7/11)	82.1% (96/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	54.0%	100%
Number of people who have passed PoE/PoC	147,864	84,592	7,555	7,836	12,699	260,546
% of travelers screened	98.0%	98.3%	98.5%	98.2%	97.0%	98.1%
% of travelers who washed their hands	97.0%	98.3%	98.5%	98.5%	99.0%	97.9%
% of travelers made aware	99.0%	98.3%	100.0%	100.0%	100.0%	99.0%
Number of alerts notified	3	1	0	0	0	4
Number of validated live alerts (suspected cases)	1	0	0	0	0	1
Number of bodies recovered today	2	1	0	0	0	3
% of suspected cases transferred to CT/CTE	0% (0/1)	N/A	N/A	N/A	N/A	0% (0/1)
% of lifeless bodies swabbed/secured	50% (1/2)	100% (1/1)	N/A	N/A	N/A	66.7% (2/3)
% of suspected cases investigated within 24 hours	66.7% (2/3)	100%	N/A	N/A	N/A	75% (3/4)

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in Tshopo (44.4%, 8/18), South Kivu (55.6%, 5/9) and Haut-Uélé (63.6%, 7/11); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of the service providers of the Nzebi PoC; low encoding of alerts in DHIS2 Tracker in Ituri; in Tshopo, 18 PoCs activated out of the 30 planned; in South Kivu, absence of isolation device at the PoEs of Kavimvira, Kalundu Port SNCC and Baraka.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 57 new positive results** (40 living and 17 deaths) out of 203 new samples received and analyzed (**positivity: 28.1%**).
- **North Kivu: 21 new positive results** (13 living and 8 dead) out of 151 samples analyzed (**positivity of 13.9%**).

- **Haut-Uélé: 9 new positive results** (8 living and 1 death) out of 45 samples analyzed (**positivity: 20%**).
- **Tshopo: 5 samples received and tested** (5 live), **all were found to be negative.**
- **Bas-Uélé: 5 samples received and tested** (4 live and 1 death), **all were found to be negatives.**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 21 of the 57 planned rings were opened (36.8%), 74 of the 74 identified households were decontaminated (100%), and all 7 health facilities were decontaminated (100%). Regarding the health surveillance system (HSS), 79 alerts were recorded, 61 HSS tests were carried out out of 79 (77.2%), and 54 of the 65 bodies were swabed (83.1%).
- **In North Kivu**, 89 rings were opened out of the 126 expected (70.6%), 28 households were decontaminated out of 131 identified (21.4%), and 70 health facilities were decontaminated out of 93 (75.3%). Four scorecard assessments were conducted with an average score of 31.6% (3 in Musienene and 1 in Kalunguta). 749 service providers were briefed, 162 community health workers were monitored and supported, 112 handwashing stations were monitored, 7 triage units were installed, and 240 liters of water were supplied to Oicha. Regarding the health and safety surveys (HSS), 56 alerts were recorded, 53 HSS were conducted, and 51 bodies were swabed.
- **In Tshopo**, decontamination of the aircraft that transported a suspected case from Buta (Bas-Uélé) and initial infection prevention and control (IPC) assessment by the Meteorological Treatment Center (90.6%); monitoring and support of 13 community health workers in 5 health zones with briefing of 142 providers on standard precautions and monitoring of 11 handwashing stations. Regarding the emergency response teams (ERT), 3 alerts were recorded, 3 EDTs were carried out, and 3 bodies were swabed (100%) at the EDT.
- **In Haut-Uélé**, 2 rings were opened out of the 5 expected (40.0%), 5 households decontaminated out of 10 identified (50.0%) and 1 out of 1 FOSA decontaminated (100%), in the ZS of Isiro and Wamba; As for the EDS, 2 alerts were recorded, 1 EDS carried out and 1 body swabed.
- **In Bas-Uélé**, 1 ring was opened out of 1 expected (100%) in Ganga; 1 EDS alert recorded and 1 swabed body.
- **South Ubangi**, disinfection of the premises of ESS frequented by suspected cases; deployment of some PCI inputs by the EIR towards the Bulu ZS.

1.4.2. Challenges

- **In Ituri** : low ring road opening rates (36.8%; 21/57); strike by decontamination teams from the Nizi ZS, for non-payment; lack of PPE in Lita.
- **In North Kivu** : Need to activate, train and motivate decontamination teams in uncovered health zones (Biena, Masereka, Kyondo, Mabalako and Vuhovi health zones); Lack of traceability of some households eligible for decontamination; Urgent need for training for all PCI Data on data collection and management tools; Non-alignment of 86.6% of PCI WASH pillar members on the INSP motivation list; Insufficient handwashing facilities in schools.
- **In Tshopo** : insufficient PCI inputs in households, schools, markets and churches; low proportion of SSE with an PCI score above 80% (2.2%; 2/92) and incinerator at HGR Makiso-Kisangani still out of order.
- **In Haut-Uélé** : lack of community PCI kits, lack of triage units in ESS and poor timeliness of daily data from health zones.

- **In Bas-Uélé :** absence of compliant waste management areas and sorting units in the 20 ESS of 5 health hotspot zones; insufficient bins and handwashing facilities, untrained IPC providers in the 5 priority health zones (Buta, Ganga, Viadana, Titule and Poko) and absence of IPC supervisors.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, 95 new admissions were recorded in the various dedicated facilities, 81 discharges (17 recoveries). Thus, 485 patients are being treated in hospital for 1015 beds (occupancy rate of 47.8%); 454/473 (96%) patients received hot meals; 9/9 (100%) children are under LNPE/SLM at the CTE, 3/3 (100%) at the nursery and 36/36 (100%) in the community.
- **In North Kivu**, 81 new admissions were recorded compared to 79 discharges, including 6 recoveries; 348 patients are hospitalized, representing an overall occupancy rate of overcapacity (152.6%); 7/7 caregivers and 41/41 frontline providers received hot meals; 1 separated child is being cared for at the daycare; 100% of separated infants; orphans (0-23 months) not breastfed and on breast milk substitute (BMS) in accordance with code: 9/9.
- **In Haut-Uélé**, 22 new admissions were recorded against 15 discharges (4 recovered). Thus, 76 patients are being treated in hospital for 136 beds, representing an occupancy rate of 55.9%; 80 accompanying persons and 32 PPLs benefited from three hot meals, except in the Rungu and Gombari health zones.
- **In Bas-Uélé**, one (1) new admission was recorded against one (1) discharge (non-case); 7 patients remain in isolation, including 1 confirmed case being treated at the HGR of Ganga; 9 high-risk contacts are under prophylaxis in Buta and Ganga.
- **In Tshopo**, One (1) new admission was recorded against five (5) discharges (non-cases). Six (6) patients are in isolation out of 31 beds, representing an occupancy rate of 19.4%.
- **In South Kivu**, there have been no new admissions. Sixteen (16) patients are in isolation out of 25 beds. resulting in an occupancy rate of 64.0%.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 45% of hospitalized patients are treated outside of ETCs, and no beds are available in Butembo and Katwa. **In Tshopo**, there is a lack of safe blood and limited technical resources. **In Bas-Uélé**, the absence of ETCs and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,460 people have been vaccinated to date out of 11,703 targeted TPL/PPL (21.0%), including 1,495 in Makiso-Kisangani, 368 in Mangobo, 331 in Kabondo, 170 in Tshopo and 96 in Lubunga.
- **In Bas-Uélé**, 500 doses of ERVEBO vaccine were received at the PEV branch in Buta and the team for issuing electronic vaccination cards arrived.
- **In Ituri**, preparations continue for the vaccination of PPL (People from Local Populations), which is scheduled to begin on the 15th September 2026 in Bunia.

1.6.2. Challenges

Low uptake of vaccination among the local population and insufficient management tools **in Tshopo**, where the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, there is no micro-plan for vaccinating the local population in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga, and Viadana).

1.7. Continuity of Care

- **In Tshopo**, 128 cases received the experimental treatment, including 16 confirmed cases and 112 under post-exposure prophylaxis (72 treatments completed, 11 interrupted, and 29 ongoing); evaluation of the Kisangani Free University Hospital Center (CHU-LIKIS), development of the recovery plan for the Betsaïda Health Center, and receipt of medications from the UG-PDSS project for the Tshopo Health Zone. All patients at the Bafwandambo Health Center are highly exposed, and only one of the 13 patients has been vaccinated; the others refused.
- **In Haut-Uélé**, the briefing of service providers on the specific treatment and prophylactic for Ebola virus disease.
- **In Bas-Uélé**, follow-up of contacts who have adhered to compassionate prophylaxis in Buta and Dingila and post-exposure prophylaxis of PPL high-risk contacts in the Ganga health zone.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 876 home visits reached 3,797 people and 43 educational talks reached 885 people; 17 mass awareness actions reached 2,415 people and 10 community debates reached 290 people; 96 community alerts were notified (including 8 deaths), 37 of the 37 feedbacks were resolved within 48 hours and 13 of the 17 community incidents were resolved, for a completeness of 14 of the 28 health zones (50%); the pillar supported the pre-listing of 394 contacts, the decontamination of 24 households, the securing of 4 bodies, the completion of 29 of the 34 EDS (85.3%) and the announcement of 22 positive results.
- **In North Kivu**, 34,737 people were reached during 9,476 home visits, 1,475 people were reached during 47 educational talks, and 4,734 during 27 mass communication campaigns; 214 community alerts were notified out of 450 expected (47.6%), 9 feedback issues were resolved out of 13 (69.2%), none of the 3 recorded incidents were resolved, and 910 people were engaged out of 4,750 (19.2%); prevention communication was conducted with seminarians at the Tumaini Letu Minor Seminary in Musienene, and 22 teachers from the Maboya Primary School (ZS of Kalunguta) and support for the community reintegration of 2 recovered patients discharged to the CTE of the Beni General Referral Hospital
- **In Haut-Uélé**, 26 educational talks reached 40 people and 14 mass activities reached 2,969 people, i.e. 40 activities and 3,009 people engaged in the health zones of Isiro, Pawa, Boma Mangbetu, Wamba, Rungu and Gombari; 14 alerts of suspected cases and deaths were notified by community actors in Isiro (8), Boma Mangbetu (2) and Wamba (4); 2 feedbacks and 2 incidents resolved within 48 hours in Isiro.
- **In Tshopo**, 476 home visits reached 1,723 people, and one educational talk reached 38 people in connection with the 27th and 28th confirmed cases in Bafwandambo; three interactive programs and 16 public service announcements were broadcast in eight media outlets, and 25 journalists in Kisangani received training; 46 community alerts were notified and all resolved, and one feedback issue and one incident were resolved within 48 hours. In Bas-Uélé, 2,712 people were sensitized in schools across six health zones, 499 people at the Onatra Beach in Aketi, 20 people attended two educational talks in Dingila, and 186 home visits were conducted, including 20 in households connected to the case of the Bambili case.

1.8.2. Challenges

Persistent community resistance: **In Tshopo**, there is a need for handwashing facilities in schools; a confirmed case who escaped from the treatment center remains at home; and there is a shortage of posters and banners. **In Ituri**, there is insufficient transportation to support other key areas, a broken printer, and no internet connection for the key area. In Haut-Uélé, there is a complete breakdown of visibility tools and communication equipment in all affected health zones. In Bas-Uélé, roads to some health zones and areas are impassable, there is a lack of transportation and phone credit, demotivation among field workers, and poor reporting from health zones.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 36 people among the new confirmed cases and 119 of the 158 confirmed cases under monitoring (75%) benefited from SMSPS interventions, as well as 37 of the 83 new

suspected cases of the day (45%) and 105 of the 147 suspected cases under follow-up (71%); 72 results announced, including 36 positive cases, 43 households supported, 37 PPLs supported, 19 children followed, 7/7 children followed in daycare and 23 of the 26 recovered followed in the community (88%); 6 of the 9 notified health zones are covered.

- **In North Kivu**, 12 people among the new confirmed cases and 197 confirmed cases under follow-up were taken care of, as well as 170 new suspected cases and 228 suspected cases under follow-up; 89 results announced, including 12 positive ones, 65 PPLs supported, 66 households accompanied and 11 of the 12 recovered are followed up in the community; 27/27 children followed up in the community and 1/1 in nursery; 373 people (102 men, 173 women and 98 children) benefited from psychoeducation sessions with the support of UNICEF and the 5 notified health zones are covered.
- **In Tshopo**, 3 confirmed cases and 6 suspected cases under follow-up benefited from SMSPS interventions at the CTE Météo; 4 laboratory results announced, all negative, psychoeducation of 9 accompanying persons at the CTE and 7 new contacts at the CT of the HGR Makiso, 2 separated children followed in community; 2 recovered have joined the response teams.

In Haut-Uélé, 38 confirmed cases under follow-up, 2 new suspected cases and 25 suspected cases under follow-up benefited from SMSPS interventions, as well as 74 accompanying persons to the CTE and 36 PPL; 41 laboratory results were announced, including 10 positive ones, 5 separated children accompanied in the community and 20 members of households and communities supported.

In Bas-Uélé, support for the confirmed case of Ganga, his family and identified contacts remains limited by the absence of a psychologist in the health zone.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10/28 health zones covered). **In North Kivu**, low coverage of interventions around the latest confirmed cases in the health zones of Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi; insufficient human resources and motivation. **In Haut-Uélé**, difficulty in the mobility of community health workers in Wamba and Boma Mangbetu; staff demotivation due to lack of payment. **In Tshopo**, absence of SMSPS providers in Wanie-Rukula and Bafwasende and low partner participation. **In Bas-Uélé**, absence of a psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- **In Bas-Uélé**: mobility ensured for the rapid intervention team deployed in the Ganga health zone and for the health zone teams responsible for verifying alerts and active search.
- **In North Kivu**: Delivery of IPC inputs to the Lubero Health Zone; deployment of two vehicles for CREC activities and investigations, two motorcycles for contact tracing in Kitsimba and the search for an escaped positive case in Lubero, and one motorcycle for transporting samples; continuation of the construction of the CTE at Matanda Hospital and start of work on the CT at Musienene General Referral Hospital.
- **In Tshopo** : monitoring of the construction of the Madula isolation facility and the Bafwasende CTE, support for the deployment of ERVEBO vaccines to Buta and Isiro and shipment of 500 doses to Bafwasende.
- **In South Ubangi**: delivery of some PCI inputs and monitoring tools for the Bulu Health Zone; deployment of an EIR.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for EDS teams for the swab of two lifeless bodies to the morgues of Ndindi and the HGR of Beni (AS of Kasanga Tuha) and for the burial of a confirmed case, as well as for IPC teams for the decontamination of the CM Sainte Ana and a household in the AS of Kanzulinzuli, a household in the AS of Mangote, the CM La Prêche in Tamende and the morgue of Ngalka in the AS of Malepe, with 100% coverage of the interventions requested; meeting of the Risk Mitigation office for the updating of the payroll list of 200 elements.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team, a lack of financial incentives for agents and non-payment of police officers and security agents assigned to guard the ESS; lack of support from security elements in several PoCs along the Butembo axis (Kangote, Kyaghala, Mususa and Komba); delayed payment of actors involved in the response and non-compliance with commitments by some partners in several health zones.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- In Ituri, 67 response staff members (57 men and 10 women) received the PSEA briefing and were sensitized in the health zones of Bunia, Nizi, Rwampara, and Mongbwalu; 579 people were sensitized in the community (170 men, 409 women, and 1 person living with a disability); two partners (WHO and IMC) reported on their PSEA activities, and 13 of the 36 health zones (36.1%) are aware of the reporting mechanism. In Bas-Uélé, 31 community health workers (CHWs) supporting the CREC pillar, including 18 women, participated in a briefing on sexual exploitation and abuse.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- The performance recorded in contact tracing over the past 24 hours (**89.3%**) and in epidemiological investigations (**98.8% of suspected cases investigated**).
- In the provinces of Ituri, North Kivu and Haut-Uélé, sustained transmission is noted while the provinces of Tshopo and Bas Uélé show sporadic notification.
- In the last 24 hours, **87 new confirmed cases**, including **26 community deaths and 9 intra-CTE deaths**, have been reported, bringing the national total to **7,345 confirmed cases and 3,545 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter of the epidemic, accounting for **77.8%** of confirmed cases and **65.5%** of new cases reported in the last 24 hours.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen preparedness and response to Ebola virus disease in the provinces of the river corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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